



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : عيبر محمد محمد زكي

العنوان : =

الحي / المدينة : النزهه

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 22-09-2026

رقم البطاقة : 26906280100428 رقم التليفون : 01273701035

العمر (تاريخ الميلاد) : 28-06-1969 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقرر بما فيه : _____

التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) قسم العلاج : الاورام

[14152]

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / _____ أ.د. / _____ أ.د. / _____ د. / _____

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

قسم : _____

جهة العلاج : _____

نفقات : _____

ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: عبيد محمد محمد زكي National ID: 26906280100428	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

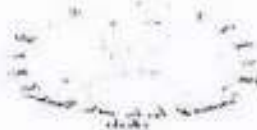
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT form

A-Patient ID:

1- Name : **تيمر محمد محمد زكي**

Sex:

3- Medical number:

Female

83155285

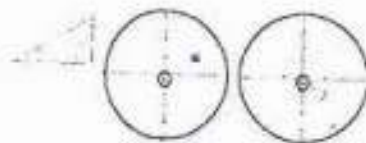
Old Med :

Do not:

5574-2020

2-ID number:

Patient Age : 56 Y



Family history

Family history

Phone

Phone

01273701035

BMI

BMI

REASON OF MDT

REASON OF MDT
+ REMOT

Marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

Menopausal
status

Menopausal status
REASON OF MDT

Initial STAGE

Initial STAGE

05/01/20

Clinical data

Breast Imaging :

Mets Workup :

CT CAP& bone scan:multiple bone+mediastinal+ LNs

BX IDC	ER 7/8	PR 5/8	Her 2 Neu 3	SISH	Ki67
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T	N	M 1 MEDIASTINA
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Sonomammography

PATIENT HISTORY

RT MRM PT2N3(12/2020)

ADJ acX4 + TAXOL X 12 + HERCEPTIN 1 YR ENDED 9/2022

on adj AI

CT CHEST:MEDIASTINAL LN

pt finished last session taxol/herceptin 29/1/2024

---> DP shifted to tykerb / aromasine

ct cap: size progression of enlarged mediastinal , hilar LN

mild progression of multiple predermal lytic osseous lesion

last mdt:fasoldex+zometa ---> refused by MOH

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for xeloda single agent + monthly zometa

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY 10/1/2024	RADIOLOGY	RADIOLOGY 10/1/2024
SURGERY	SURGERY 10/1/2024	PATHOLOGY	PATHOLOGY 10/1/2024
Psychiatric	Psychiatric	Nutrition	Nutrition



Pathology Report

Name: عبيد محمد نكي

Age: 50 years.

Clinical data: Right breast mass.

Specimen type: Right MRM

Date(received) : 26/12/2020

Date(reported) : 4 /1/2021

Lab. Number: 2157

Hospital Number: 5574 /2020 نفقة دولة

Gross Description:

Right MRM specimen : measured 20x16x6cm, covering skin 20x14cm, axillary fat 8x4cm, nipple was unremarkable , cut section revealed UOQ ill-defined firm greyish white mass 2.5x2x1cm, away 3cm from skin and 2cm from deep margin, axillary fat dissection revealed 20 nodules up to 3.5cm.

Microscopic Examination

Sections examined revealed breast tissue infiltrated by invasive , malignant tumor formed of nests and cords of moderately anaplastic ductal epithelial cells with desmoplastic stroma.

The examined axillary LNs showed heavy metastatic deposits in 18 nodes (18/20) with extra capsular extension.

Diagnosis:

Right breast mass, Right MRM.

Invasive duct carcinoma grade II.

Minimal DCIS .

Positive lymphatic tumor emboli.

Negative perineural invasion.

Free skin, nipple & margins .

L.Ns status(18/20) with extra capsular extension.

Stage p T2 p N3a.

Prof. Gamal Aba L. Aty.
Dr. Enas Mohammed Ali

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 74/2025	Pat. ID: 20109

Patient Name: Abeer Mohamed Mohamed Zaky

Date: 29-7-2025

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

HISTORY : A known case of breast cancer underwent right mastectomy .As compared to the previous report dated 17/04/2025,the current study reveals:

FINDINGS:

- Evidence of right mastectomy with clear operative bed
- Still noted stable middle lung lobe subpleural reticulations mostly representing post irradiation sequel .
- Still noted bilateral basal & middle subpleural atelectic bands are seen
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses or consolidative patches
- Stationary course as regards prevascular heterogenous nodal lesion with internal tiny foci of calcifications measuring about 4.7 X 2.9 cm.
- Still noted enlarged mediastinal and bilateral hilar lymph nodes ,the largest is subcarinal location measruing about 2.4 x 3.7 cm as compared to in the previous study 2.6 x 4.4 cm.
- Normal C.T. appearance of the mediastinum vessels.
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- Still noted multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton with lytic burden more at the manubrium sternal lesion.

CONCLUSION :-

Follow up study with overall stationary course of the disease.



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 74/2025	Pat. ID: 20109

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS-:

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

- Normal C.T. density of the liver parenchyma, with no focal lesions .
- No extra or intra-hepatic biliary dilatation .
- Both kidneys are functioning and normal in dimensions.
- The spleen and pancreas show normal C.T. appearance .
- Normal size and C.T. density of the adrenal glands .
- No ascites .
- The urinary bladder shows normal size and wall thickness with no calculi inside.
- The uterus is of normal C.T. appearance .
- Normal ischio-rectal fossae on both sides .
- No sizable abdominal or pelvic lymph glands.
- **Still noted** multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton .



IMPRESSION:

Overall stationary course of the disease

Much Obligated,

Mona Ali,MD

Not Approved

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 15/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 5574/2020	Pat. ID: 20109

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

HISTORY: A known case of breast cancer underwent right mastectomy .As compared to the previous report dated 26/7/2025,the current study reveals:

FINDINGS:

- Evidence of right mastectomy with clear operative bed
- Still noted stable middle lung lobe subpleural reticulations mostly representing post irradiation sequel .
- Still noted bilateral basal & middle subpleural atelectic bands are seen
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses or consolidative patches
- Stationary course as regards prevascular heterogenous nodal lesion with internal tiny foci of calcifications measuring about 3.4x4.4 cm was 4.7 X 2.9 cm ?granulomatous.
- No evidence of sizable mediastinal or hilar lymph nodes.
- Normal C.T. appearance of the mediastinum vessels.
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- Still noted multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton with lytic burden more at the manubrium sterni lesion.



CONCLUSION :-

Follow up study with overall stationary course of the disease as regards previous report.

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS:-

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

- Normal C.T. density of the liver parenchyma, with no focal lesions .

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 17/08/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 56 Year(s)
Medical ID: 5574/2020	Pat. ID: 20109

MRI OF THE BRAIN WITH CONTRAST

Findings:

- No cerebral parenchymal areas of abnormal signal intensity.
- no evidence of enhancing cerebral or meningeal lesions.
- Normal size and configuration of the ventricular system with no evidence of deformity or displacement.
- No cerebellum or brain stem areas of abnormal signal intensity
- No mid line shift.
- Examined PNS are unremarkable .

OPINION:

known case of breast cancer:

- No evidence of metastasis. .
- No recent ischemic insult.

Dr. Reham Khedr (MD)

Signature Reeham.Khader
19/08/2025 7:24 PM



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 23/04/2024
Exam : C.T CERVICAL SPINE	Age : 55 Year(s)
Medical ID: 5574/2020	20109

MRINON CONTRAST CT OF THE WHOLE SPINE

FINDINGS:

- Mild progressive course of the right iliac bone lytic osseous lesions.
- Otherwise stationary other bilateral pelvic bones and few vertebral predominately lytic osseous lesions
- Normal dimensions of the spinal canal.
- No evidence of vertebral collapse.
- Normal size of the conus medullaris.
- No para-spinal soft tissue masses or collection.

OPINION:

- Mild progressive course of the right iliac bone lytic osseous lesions.
- Stationary rest of the study.

Dr. Ali Saber, MSc.

Signature Prof. Menna Tallah Hanafy

30/04/2024 12:16 م



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 09/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 5574/2020	

LEFT BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had right MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- fatty parenchyma of the left breast (ACR a)
- No speculated masses or suspicious micro calcific clusters of the left breast.
- Complementary ultrasound showed no left solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts of the left breast.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions.
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- Essential normal study of the left breast (BIRADS 1)

Best regards,
Dr/Asmaa Zeinhom, Msc

Signature Temp
09/02/2023 2:03 PM



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 23/10/2024
Exam : Item 1	Age : 55 Year(s)
Medical ID: MOSTAFA^BRAIN	Pat. ID: 20109

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1WIs
- Post contrast series

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal, yet a left cerebellar small subcentimeteric lacunar infarct noted (displaying fluid signal) with no surrounding mass effect.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

- No definite enhancing intra cranial masses.
- A left cerebellar small old lacunar infarct.

Dr. Sherif Sameh, M.D.

Health Insurance Organization
Nasr City Cancer Center
(NCCC)



شركة التأمين الصحي
مركز اور او مدينة نصر

اسم المريض : حبيب محمد محمد

تاريخ الخدمة : 2020/10/27

Pathology Report

Lab.No:20/811

Specimen:

Right breast mass.

Gross:

six cores, each 1.2 cm long, totally submitted.

Diagnosis:

Right breast mass, tru cut biopsy:

-Invasive duct carcinoma grade II

With DCIS (cribriform, solid pattern in 30% of limited biopsy)

Prof.Dr.Hussain Elsayed

Doc: 11/11/2020 Time: 14:14:14



DMS
مدى



جمهورية مصر العربية
بطاقة تحقيق الشخصية

عبر
محمد محمد زكى
٢٦ ش احمد فهد بيومى مصر الجديدة
النزهة - القاهرة

١٩٦٩/٠٦/٢٨ ٢٦٩.٦٢٨.٠١.٠٠٤٢٨
KC2789262

٢٠٢٢/٠٨ ٢٦٩.٦٢٨.٠١.٠٠٤٢٨

ربة منزل
أنثى
متزوجة
مسلمة
باتقا الفكى محمد باتقا
البطاقة سارية حتى ٢٠٢٠/٠٨/٠١

١

جوسافي
رونياني
٨١

٩ - ١٧

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة المصرية
للتنظيم
الأسري

MDT FORM

09/16/2026

Name:	Abir Mohammad Zaky Mohammad	MRN:	126
Age:	57 Y	ID:	26906280100428
Menopausal Sta		Phone:	01210939545
NRS SCORE:	ps 3	Marital Status:	married
BMI:		Family History:	
CO Morbidity:	-ve	Reason Of MDT:	mbc

Clinical Data:

BX:

ER: 7/8

PR: 5/8

HER2: +3

KI67:

Sonomammography:

Mets Workup:

2020 :rt mrm (t2 n3)
adjuvant : ac*4+taxol *12+herceptin 1 year (till 1/2024)
2024:tykerb +aromasin
1/2025 :single agent xeloda till now
8/2026 CT CAP :
1/Multiple hepatic, pulmonary and osseous metastatic lesions.
2/Progressive course since the last study dated 26-7-2026
8/2026 bone scan :
**Stationary widespread osseous metastases, showing no significant interval progression

8/2026 MRI BRAIN :
Bilateral cerebellar few small foci of CSF signal, likely old ischemic Partial foci
empty sella
Prominent extra axial CSF spaces (age related)
• No acute brain infarction
•No evidence of gross SOL or mass effect on non contrast base

History:

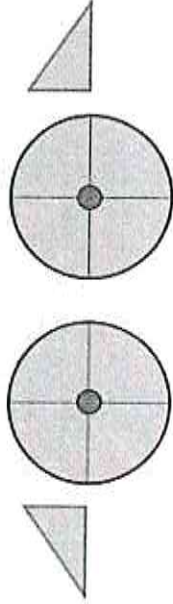
Recommendations:

MDT Outcome & Plan

For:
1.Re-do MRI brain with contrast
2.Start Taxol/Carbo weekly + Herceptin (If not approved by MOH, trial through NGO)

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. امينة عيسوي				

١٦/٩/٢٠٢٦
Omar
16/9/2026